CROUP

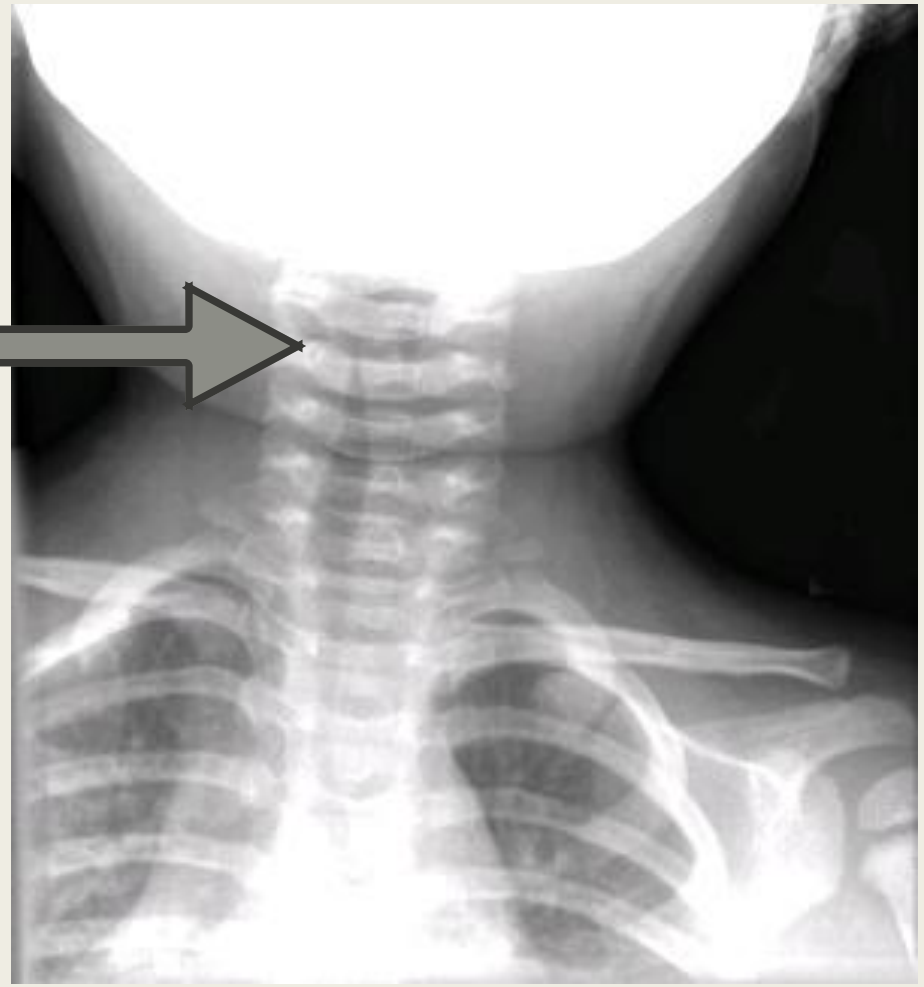
- Laryngotracheitis
- Inspiratory stridor, cough, hoarseness of voice.
- Characterized by “barking cough”
- Viral croup = Classic croup. Coryza and fever.
  6 months – 3 years
  self-limiting. Resolves in about 3 days.
- Spasmodic croup = Frequently recurrent croup
  6 months – 3 years
  self-limiting. Resolves fast. Recurrent
  usually runs in the family
- Viruses: RSV, influenza, parainfluenza, SARS-CoV-2, measles.

WESTLEY SCORE

Clinical feature	Assigned score
Level of consciousness	
Normal, including sleep	0
Disoriented	5
Cyanosis	
None	0
With agitation	4
At rest	5
Stridor	
None	0
With agitation	1
At rest	2
Air entry	
Normal	0
Decreased	1
Markedly decreased	2
Retractions	
None	0
Mild	1
Moderate	2
Severe	3
Total	

Score total	Severity	Description	Management
≤2	Mild	Occasional barky cough, no stridor at rest, mild or no retractions	▪ Home treatment – Symptomatic care including antipyretics and oral fluids ▪ Outpatient treatment – Single dose of oral dexamethasone* 0.15 to 0.6 mg/kg (maximum 16 mg) or oral prednisolone (1 mg/kg)
3 to 7	Moderate	Frequent barky cough, stridor at rest, mild to moderate retractions	▪ Single dose of oral dexamethasone 0.6 mg/kg (maximum 16 mg)* ▪ Nebulized epinephrine¶ ▪ Hospitalization is generally not needed but may be warranted for persistent or worsening symptoms after treatment with glucocorticoid and nebulized epinephrine
8 to 11	Severe	Frequent barky cough, stridor at rest, marked retractions, significant distress	▪ Single dose of oral/IM/IV dexamethasone 0.6 mg/kg (maximum 16 mg)* ▪ Repeated doses of nebulized epinephrine¶ may be needed ▪ Inpatient admission is generally required unless marked improvement occurs after treatment with glucocorticoid and nebulized epinephrine
≥12	Impending respiratory failure	Depressed level of consciousness, stridor at rest, severe retractions, poor air entry, cyanosis or pallor	▪ Single dose of IM/IV dexamethasone 0.6 mg/kg (maximum 16 mg) ▪ Repeated doses of nebulized epinephrine¶ may be needed ▪ Intensive care unit admission is generally required ▪ Consultation with anesthesiologist or ENT surgeon may be warranted to arrange for intubation in a controlled setting

Diagnosis & Treatment

- Made clinically.
- Labs are of no value. Agitation → worsens croup
- Radiographs not required.
- Treat according to westly score.

STEEPLE SIGN